

Neck Injury (NEXUS Criteria) - Female (Adult)

Algorithm 1046 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only adult blunt-trauma cervical-spine pathway describing the NEXUS low-risk criteria and routing for in-person examination; it must not clear the cervical spine by telephone.
- Adult patient.
- Female.

Clinical background

- NEXUS classifies a blunt-trauma patient as low probability only when all five clinician-assessed findings are absent: posterior midline cervical tenderness, focal neurological deficit, altered alertness, intoxication and a painful distracting injury. It supports an imaging decision; it does not diagnose every injury or replace clinical judgment. Telephone assessment cannot reliably perform these examinations, so this pathway never clears the neck remotely. It is adult-only here; children require a separate approved pediatric pathway, and older adults warrant particular caution.

Pain severity

- Mild (1-3): Does not interfere with normal activity, movement, concentration, or sleep.
- Moderate (4-7): Interferes with normal activity, movement, concentration, or sleep.
- Severe (8-10): Prevents normal activity or movement, or is described as excruciating.

Initial assessment

1. [OPEN_TEXT] How did the neck injury happen?
2. [DURATION] When did it happen?
3. [YES_NO] Any numbness, tingling, or weakness in the arms or legs?
4. [PAIN_SEVERITY] What is the current pain severity from 0 to 10, and how is it affecting normal activity or movement?
5. [PREGNANCY] Is the patient pregnant or possibly pregnant? If yes, record gestation where known.

Triage and redirect questions

1. Disposition level 70

Is the neck pain non-traumatic, with no recent blunt injury, collision, fall, or impact?

NEXUS was validated for blunt trauma and must not be applied to non-traumatic neck pain.

1. Disposition level 100

Is there weakness, numbness, or paralysis in the arms or legs, loss of bladder or bowel control, or is the patient unable to move part of their body since the injury?

Universal emergency rule-out added ahead of the NEXUS Criteria itself (not part of the published rule) - a focal neurologic deficit is itself one of the 5 NEXUS criteria and always the highest-urgency finding, so it is elevated to the emergency screen here.

1. Disposition level 70

On in-person examination, is there posterior midline cervical-spine tenderness?

Published NEXUS midline tenderness criterion (Hoffman et al., 2000). A positive answer to ANY NEXUS criterion means imaging is indicated.

2. Disposition level 100

Is alertness abnormal or uncertain, including confusion, disorientation, delayed response or inability to participate reliably?

Published NEXUS altered-consciousness criterion.

3. Disposition level 70

Is there evidence or uncertainty about intoxication by alcohol, drugs or medicines that could impair alertness or pain perception?

Published NEXUS intoxication criterion.

4. Disposition level 70

Is there a painful distracting injury, unreliable examination, high-risk clinical concern, pregnancy, or age 65 years or older?

Painful distracting injury is a published NEXUS criterion. Age, pregnancy and other clinical concerns are not additional NEXUS points; they are flagged for cautious in-person assessment and individualized imaging decisions.

1. Disposition level 70

Even if every criterion is reported absent, has the patient not had all five NEXUS findings assessed by a qualified clinician in person?

All five NEXUS findings must be assessed in the intended clinical setting. Telephone reports cannot establish a validated rule-negative result or authorize removal of spinal precautions.

Advice

Emergency neck injury precautions

- Call Qatar emergency services on 999. Ask the patient to remain still in the position found unless there is immediate danger, breathing must be managed, or the 999 call-taker instructs otherwise. Do not test neck movement, remove a fitted collar, or allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening weakness or numbness; new loss of bladder or bowel control.

Positive or uncertain NEXUS finding

- Arrange prompt in-person trauma assessment and clinician-directed imaging. Minimize unnecessary neck movement. Pregnancy must be disclosed, but should not delay emergency stabilization or necessary imaging; imaging risk-benefit decisions belong to the treating team.
- Call Back If: new numbness, tingling, or weakness; worsening pain.

Telephone assessment cannot clear the cervical spine

- Arrange in-person examination. Do not use a remotely reported negative checklist to remove spinal precautions or advise home care. The exact non-emergency Qatar destination, transport and analgesia pathway are local approval required.
- Call Back If: new numbness, tingling, or weakness develops; pain worsens.

First aid

- Call Qatar emergency services on 999. Ask the patient to remain still in the position found unless there is immediate danger, breathing must be managed, or the 999 call-taker instructs otherwise. Do not test neck movement, remove a fitted collar, or allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hoffman JR, Mower WR, Wolfson AB, Todd KH, Zucker MI. Validity of a set of clinical criteria to rule out injury to the cervical spine in patients with blunt trauma. National Emergency X-Radiography Utilization Study Group. N Engl J Med. 2000;343(2):94-99.
- American College of Radiology. ACR Appropriateness Criteria: Acute Spinal Trauma. Revised 2024.

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source clinical decision-rule content derived from the cited peer-reviewed publications; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Telephone adaptations, local disposition mappings, and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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